National Cancer Control Programme (NCCP) 00455 · clinician draft

Pembrolizumab · every 3 weeks · melanoma before and after surgery

Your diagnosis and treatment setting Selected stage IIIB melanoma eligible for the NCCP adjuvant indication: the treating clinician may choose pembrolizumab 200 mg before and after surgery. This is a distinct, discretionary perioperative plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Before surgery

Active medicines: Pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic or premedication is specified for this monotherapy. Any symptom-treatment medicine requires an individual authorised prescription.

Additional phase-specific support

- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
- Discuss fertility before starting. Use effective contraception during treatment and for at least 4 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself.
- Immune adverse effects may require corticosteroids and/or long-term hormone replacement under specialist direction. Do not stop prescribed treatment yourself.
- This is an incomplete clinical/pharmacy review draft, not an issued prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00455 v16 · p5–7 · NCCP 00455 v16 and local supportive-care policy

Sources checked 2026-11-06 · SACTCheck v0.84.41 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

After surgery

Active medicines: Pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic or premedication is specified for this monotherapy. Any symptom-treatment medicine requires an individual authorised prescription.

Additional phase-specific support

- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
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